

SECTION A – BASIC DETAILS

Country Team: India

Team Name: India 2.0

Team Members:

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SECTION B – PROBLEM IDENTIFICATION

- Why this problem?

Early Diagnosis and screening were our focus area because majority of the cases the rare diseases has a delay of around 5-7 years in India and can even go up in many cases. Even at times misdiagnosis leads to wrong treatment causing more agony to patient and caregivers, along with financial burden on the family. An Ai enabled system will help speed up the process of identification while awareness takes place in the field through asha workers

and NGOs making well informed referral system to ease pain points in diagnosis and help leverage tech in reducing diagnosis time.

(Briefly describe why your team chose it — relevance, urgency, local applicability.)

- Who is affected?

This Ai based system immediately benefits the Patients (Individually & Patient advocacy groups), Families, Health system becomes more streamlined from symptoms to diagnosis timeline and Clinicians get better organized data to initiate their investigation.

(Specify the beneficiary group: patients, families, clinicians, health system, etc.)

- Current gaps or challenges in the existing system:

System at present lacks a proper channel for a patient to get faster diagnosis due to lack of awareness in remote areas about rare diseases, this is even common in urban areas thus our system looks to bridge these gaps while encouraging transparency in diagnosis and collaboration among the healthcare practitioners and the system keeps evolving with AI. Also, patient data is fragmented leading to further delay.

(Diagnostic delay, lack of data, poor coordination, low awareness, etc.)

SECTION C – PROPOSED SOLUTION

- Name / Working Title of Solution: **PATHFINDER: AI-Powered Symptom Navigator & Integrated Care Coordination Platform**
- Is the solution digital, partially digital, or non-digital?
It is digital powered by AI
- What does your solution aim to do?
AI-driven platform that dramatically shortens the diagnostic journey for rare disease patients by intelligently navigating complex symptom landscapes, connecting users to vetted specialists and diagnostic

resources, and maintaining a secure, portable health narrative. By leveraging multilingual NLP, phenotype mapping, and geo-localized care networks.

(Two–three lines describing the core idea.)

- How will it function in simple terms?

It will help matchup the symptoms with the database close to the disease or group of diseases and then help with suggestions to nearest centres/clinicians based on the geo location of the patient. Further keeps learning with each referral and confirmed diagnosis building the database of symptoms and diseases.

(Bullet points or a short paragraph)

- Who are the primary users?

Patients. Patient Groups, Community health Workers (Asha workers) and Clinicians

(Patients, HCPs, community health workers, policymakers, labs, etc.)

- What is the unique value or innovation?

It is First multi-modal platform integrating NLP, phenotype mapping, specialist networks, and peer support in a unified UX. Simplifying the process for early diagnosis and better data management.

(Explain why it is different from existing approaches.)

SECTION D – IMPACT & SUSTAINABILITY

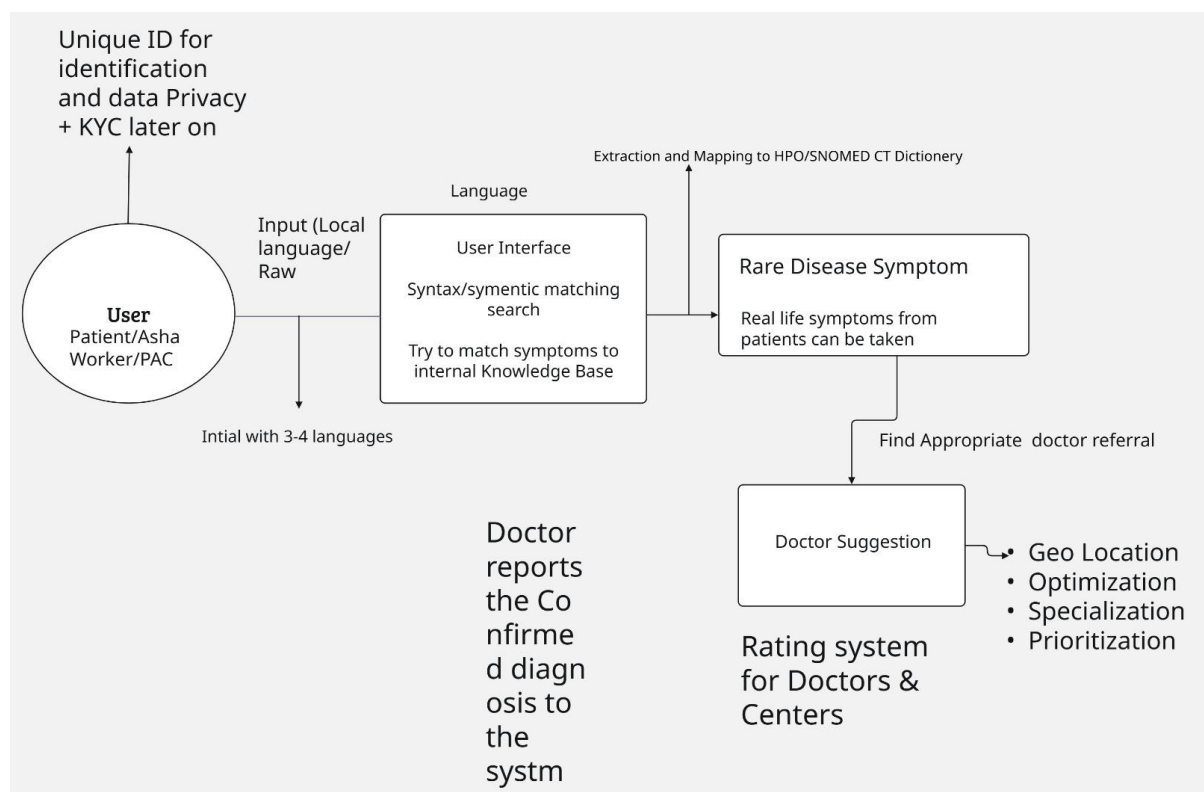
- Expected impact:

Will help improve awareness, speed up the diagnosis, smoothen the referral system for patients and better data management for future health issues.

(e.g., faster screening, improved coordination, better awareness, etc.)

- How could this solution be integrated into real-world systems (healthcare, policy, community)?

This can be integrated with already existing healthcare system upgrading it and also at community levels for better coordination between various healthcare centres/ clinicians.



Whiteboard Rough Diagram of flow (This will be further enhanced)